

Gastric tube necrosis and reconstruction with colon interposition

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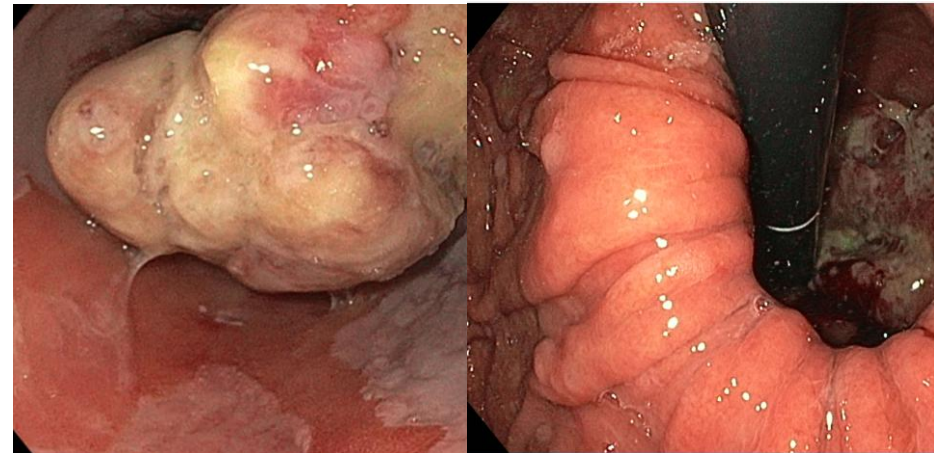
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Background and referral reason

- **75-year-old woman** referred in Nov 2021 with suspicion of **cardia cancer**
- Past medical history: **Hypertension, rectopexy x2**
- **Very active lifestyle**, gym and dance instructor for seniors
- **Non-smoker**
- Had been **under investigation by her GP** for anemia

Background and referral reason

- **Gastroscopy (30 Nov 2021): Tumor in cardia, Siewert type II**
- Referral via “**Standardized Care Pathway**”
 - ☐ National Program organized by **Regional Cancer Centers**
 - ☐ Ensures **rapid and equitable diagnosis and treatment** of suspected cancer
 - ☐ Each cancer type has defined **alarm symptoms** and target **time frames**
- **PET/CT** requested as part of the diagnostic work-up



MDT (14 Dec 2021)

- Poorly differentiated **AC**
- Her2-neg, **MSI-H**
- **cT3N0M0**
- **Recommendation: Neoadj chemotx**



Neoadjuvant treatment outcome

- **Regimen:** FLOT x 4
- **Completed:** 8 Feb 2022
- **Tolerance:** very good
- **Response:** Partial response of the primary tumor
- **Restaging:** No new findings
- **MDT recommendation** (22 Feb 2022): Robot-assisted McKeown esophagectomy

Operation

- **Date:** 24 March 2022
- **Procedure:** Robot-assisted McKeown esophagectomy
- **Course:** Uneventful, technically straight forward
- **Duration:** 8 hours
- **Estimated blood loss:** 50 ml

Early Postoperative course (POD 3)

- **Location:** High Dependency Unit
- **Clinical picture:**
 - ☐ Patient **unwell, febrile**, dyspneic and in **respiratory distress**
 - ☐ Reported **distinct pain in the left chest**
- **Initial management:**
 - ☐ **iv antibiotics**
 - ☐ **CT-scan performed**
- **CT-findings:**
 - ☐ **Herniation of colon** in the left hemithorax
 - ☐ Minor cervical **anastomotic leak**



Emergency reoperation

➤ **Initial approach:** Laparoscopic exploration

- ☐ Colon herniated through the hiatus into the thorax
- ☐ Reduction unsuccessful, conversion to laparotomy

➤ **Open findings and management:**

- ☐ Phrenotomy performed
- ☐ Colon reduced into abdomen, no ischemia observed
- ☐ Intraoperative gastroscopy: 4-5 cm necrosis of the proximal gastric conduit, likely secondary to the compression from the herniation
- ☐ Resection of necrotic conduit segment
- ☐ Attempt to reach the neck unsuccessful despite Kocher mobilization
- ☐ Hiatus closed and cervical esophagostomy performed
- ☐ Gastrostomy performed

➤ **Operation time:** 4 hours

Postoperative course

- **Initial stay:** ICU x 3 days
- **Recovery:** Rapid and uneventful
- Transfer to surgical ward after stabilization
- Good general state, mobilized and tolerating enteral feeding
- **Discharge:** To rehabilitation unit on 6 April 2022

Preoperative MDT (29 Sept 2022)

➤ **CT-scan:**

- ☐ No signs of recurrence
- ☐ Good vascular supply to both right and left colon

➤ **MDT decision:** Proceed with reconstruction using colon interposition



Reconstruction (11 Oct 2022)

- **Colon mobilization:** thorough exploration performed
- **Vascular assessment:**
 - ☐ **Right colon** superior blood supply, chosen for reconstruction.
 - ☐ **Ileocolic artery** divided (after 30 min test-clamp).
- **Transection:**
 - ☐ Distal ileum
 - ☐ Transverse colon just distal to hepatic flexure
 - ☐ Middle colic artery preserved
 - ☐ Interponate perfused via right colic artery.
- **Route:** Substernal pull up of the interposed colon
- **Anastomoses:**
 - ☐ Cervical side-to-side esopgago-ileic
 - ☐ Side-to-side cologastric
 - ☐ Sise-to-side ileocolic
- **Pyloromyotomy** and **feeding jejunostomy** performed

Postoperative course *after reconstruction*

- **Initial stay:** HDU
- **Clinical findings:** Signs of delayed emptying of the interposed colon
- **CT-scan:**
 - ❑ Distended colonic interponate
- **Management:** Insertion of nasogastric tube
- Otherwise uneventful recovery



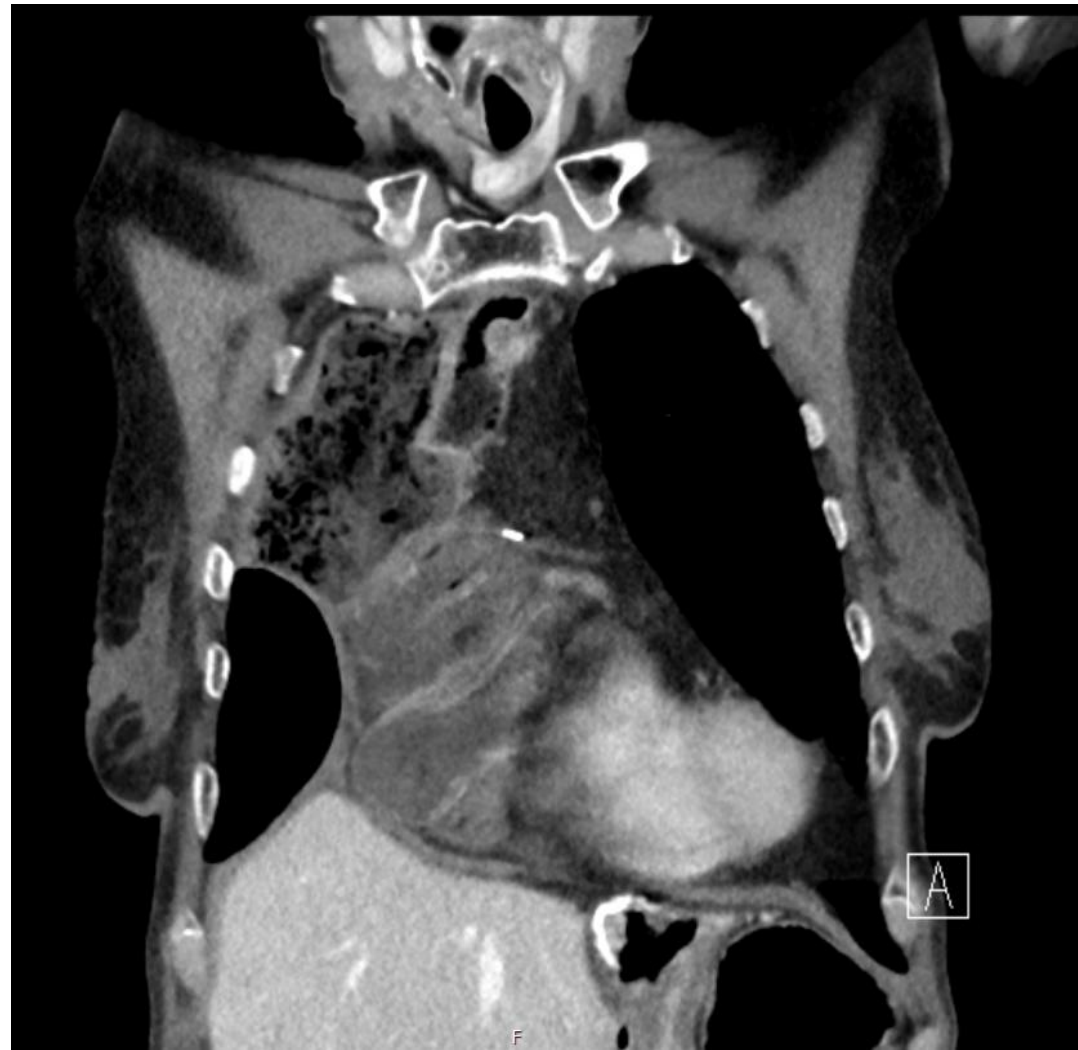
Postoperative course *after reconstruction*

- **CT POD 12**
 - ❑ Colonic interponate still distended
 - ❑ **But** contrast passage
- **Gastroscopy POD 16:** no stricture detected
- **Good general condition**, fully mobilized
- **Discharged** to rehabilitation unit POD 18



Follow-up

- **Last outpatient visit:** 1 Oct 2025
- **CT-scan:**
 - ❑ No evidence of recurrence
- **General condition:** Feeling excellent
- **Lifestyle:** very active, organizes several dance courses for seniors
- **Nutrition:** good appetite, stable weight, no dysphagia or nausea



Discussion points

➤ Choice of conduit

- ☐ Main factors influencing the selection of right versus the left colon for reconstruction

➤ Timing of reconstruction

- ☐ Pros and cons of immediate vs delayed reconstruction in an emergency setting

➤ Prevention of herniation

- ☐ Closure of hiatus?
- ☐ Colonic fixation?

THANK YOU!

